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English 2
12 April 2026

Large Inquiry Question: After considering Obama's Eulogy, what is prompting the increase in depression, mental health issues, and suicide in our country?

Annotated Bibliography

Buechner, Hadassah, et al. "Community interventions for anxiety and depression in adults and young people: A systematic review." *Australian & New Zealand Journal of Psychiatry* 57.9 (2023): 1223-1242.

Hadassah Buechner and colleagues argue in "Community Interventions for Anxiety and Depression," that U.S. healthcare systems have exceeded their ability to address the growing mental health crisis, and that community-driven alternatives, including music, arts, sports, and gardening, offer relief for depression and anxiety. The authors took randomized controlled trials and concluded that individuals who engaged with community assets produced clinically significant reductions in depression and anxiety symptoms across adult populations. Buechner et al. note that while all their reviewed studies reported decreases in anxiety or depression outcomes, the quality of the individual studies fell within a wide range, underscoring the need for further research. They also describe social prescribing – the practice of linking individuals to community resources through primary care – as an effective, but underdeveloped model. The authors push for community involvement, stating that, in some cases, a cost-effective standalone approach that reduces dependence on emergency and hospitalization services. Particularly in underdeveloped or lower-class neighborhoods, the idea could be used as a supplement where clinical options are scarce.

Are there measurable differences in mental health outcomes between communities that contain churches and social hubs versus those without?

Buechner and colleagues provide scientific support for what Obama captures in the moral terms of his eulogy: the social and civic infrastructure of a community is not extra, but a health necessity. Obama shows grief to communities that have grown passively and have disconnected from the greater society, allowing members who are suffering to disappear into silence. This research confirms what is lost when infrastructure disappears. The authors find that community-based activities are used as a social cure for mental health, a concept that connects directly to Obama's argument that communal bonds – the churches, gathering areas, etc. – are the first defense against individuals' depression and suicide. Buechner et al. write that "healthcare systems do not currently have the capacity to deliver interventions to treat or prevent mental illness to all who would potentially benefit" (Buechner et al. 643), which is the missing gap that social hubs are positioned to fill. When communities have access to active churches, neighborhood centers, and recreational groups, individuals who would never seek help within the healthcare system find themselves inside of networks that reduce social isolation and provide informal support. The growing evidence this source synthesizes suggests the ruination of these community spaces, which Obama identified as a spiritual and civic failure, has a direct cost in the declining depression outcomes. The source answers the small inquiry question with evidence

while simultaneously strengthening the answer to the large inquiry question: the rise in depression, mental health crisis, and suicide in America is partly caused by allowing the communal infrastructure to fall apart.

Crockett, Marcelo A., et al. "Interventions to reduce mental health stigma in young people: A systematic review and meta-analysis." *JAMA Network Open* 8.1 (2025): e2454730.

Marcelo A. Crockett and colleagues created a systematic review and analysis of 97 randomized controlled trials involving 43852 people between the ages of ten and twenty-four to see the effectiveness of interventions directed towards reducing mental health stigmas and improving individuals' willingness to seek help. Their search is based on five major academic databases – CENTRAL, CINAHL, Embase, PubMed, and PsycINFO – and uses information through February 2024, making it a very current review. Crockett et al. find that interventions of anti-stigma produce short-term improvements in young people, but the evidence for long-term effectiveness remains small, as longer studies are needed. The authors base their work on the premise supported by the wider literature: mental health stigma is a significant barrier for help-seeking young people, often shown through internalized shame, self-labeling, self-discrimination, and the belief that mental illness reflects personal weakness rather than treatable illnesses. The review also touches on how self-stigma – internal negative societal beliefs about mental illness – is linked to worsening mental health conditions, lower quality of life, and a reduced chance for victims to seek help among adolescents and young adults

In what ways does the stigma of mental illness prevent young adults from seeking early help?

Crockett and colleagues' analysis is one of the most current and complete scientific statements on the relationship between stigma and help-seeking behaviour in young adults, and its findings reveal the failure that Obama's eulogy mourns in moral terms. Obama speaks about a culture in which suffering is expected to remain invisible or blend in, in which communities have grown blind to the pain of their neighbors, and in which the most vulnerable people retreat into silence rather than risk being exposed. Crockett et al. reveal the idea that produces this silence in clinical terms: stigma, and especially self-stigma, in which young people absorb and apply society's negative judgements to themselves. This is a barrier powerful enough to overpower both suffering and available resources. The analysis looks at 97 trials and nearly 44,000 young people's stories to collectively show that the default condition for young people is mental stress and anxiety. Crockett et al. confirm that "mental health stigma is a considerable barrier to help-seeking among young people" (Crockett et al. e2454730), an idea that becomes a metaphoric death sentence at a population level when young adults who are depressed and suicidal choose to suffer in silence rather than risk the social cost of seeking help. The connection to the large inquiry question is direct: when the cultural environment teaches young people that mental illness is a mark of weakness and failure, the crisis that Obama speaks of does not require a single act of violence to deepen – it requires everyone to stay quiet, and the evidence shows that most do.

Shannon, Holly, et al. "Problematic social media use in adolescents and young adults: systematic review and meta-analysis." *JMIR mental health* 9.4 (2022): e33450.

Holly Shannon and colleagues examined eighteen studies with a total of 9,269 participants to find the relationship between social media use and mental health outcomes in adolescents and young adults. The analysis showed moderate and significant correlations between social media and depression, anxiety, and stress. The authors defined this use of social media as problematic use, addiction-like behaviour where individuals lose self-control, preoccupation, mood alteration, and withdrawal symptoms associated with social media. The study finds no variation in results between different ages, genders, or years of publication, suggesting that the problematic complications associated with social media use are consistent across all demographic groups rather than being confined specifically to fragile subsets of groups. Shannon et al. discuss the need for further research into the specific psychological functions that drive these associations – whether harm comes from online shaming, social comparison, the displacement of face-to-face interactions, or intrusions on user sleep – while labeling the overall patterns as harm already too toxic to dismiss.

How has the increase in online shaming and social media use contributed to a sense of isolation mentioned in Obama's speeches?

Shannon and colleagues provide quantitative evidence of the harm that Obama speaks about in civic language. The replacement of human face-to-face connections with digital engagement has produced psychological damage on a global scale. Obama speaks of the modern comfort level with silence, that the modern generation tends to retreat into passive lifestyles rather than experiencing the difficulty and vulnerability of a growing community. This research shows the cost to an individual's mental health when they retreat into the digital, isolated world. Social media platforms like to behave as arenas for social comparison, public shaming, and cyberbullying that deepen the isolation they strongly claim to resolve. The evidence that these harms are consistent across age groups and genders – that the “meta-regression” shows “moderate but statistically significant correlations between problematic social media use and depression” (Shannon et al. e33450) – disproves all arguments that only vulnerable communities or individuals are at risk. The source answers the small inquiry question by documenting the exact psychological processes from the encoding of online shaming and social media use to the almost learned helplessness that comes from repeated exposure, and it connects to the large inquiry question by labeling the digital environment as one of the largest drivers of the mental health crisis that Obama's eulogy solidifies. The crisis is a psychological battle that is caused by the social ideals and cultural assumptions created by modern civilization: society has allowed a set of manipulative platforms to replace social hubs that once kept vulnerability high and harmful psychological processes low, without building any civic or regulatory institutions to protect social media users.

Kupferberg, Aleksandra, and Gregor Hasler. "The social cost of depression: Investigating the impact of impaired social emotion regulation, social cognition, and interpersonal behavior on social functioning." *Journal of Affective Disorders Reports* 14 (2023): 100631.

Aleksandra Kupferberg and Gregor Hasler argue in “The Social Cost of Depression” that depressive disorders are both understood as clinical conditions and social ones, characterized by impairments in social functioning stronger than those experienced by others suffering other medical conditions. The authors review evidence from three sectors: self-focused social

cognition, which refers to an individual's self-image in social situations; other-focused social cognition, which refers to the ability to read and understand others' emotions; and interpersonal behavior, which includes the full range of social engagement from both conversations and relationships. They found that individuals with depression show reduced capacities for empathy, emotional recognition, and social battery – not because they are indifferent, but because their emotional processing is blocked, making social engagement a true struggle. Kupferberg and Hasler discovered that social performance is inversely related to depression severity, meaning that as a person's depression deepens, their social performance decreases proportionately. The authors also touch on empathetic distress, where depressed individuals withdraw from social interactions due to an overwhelming inability to tolerate the emotional weight of another person's concern, added on top of their own existing suffering.

What role does “social apathy” play in the processes of emotional recovery?

Kupferberg and Hasler provide the clinical framework needed to understand Obama's ideas of social blindness in his eulogy – the tendency for individuals to grow numb to suffering and turn away from the pain rather than attempting to seek help and fix it. Obama identifies this numbness as a systematic pattern and learned response to environments in which suffering is normalized to appear strong and happy to others. In other cases, this pattern comes from the inaccessibility for individuals to seek or receive help due to cultural expectations they feel they must uphold for themselves and their peers. Kupferberg and Hasler reveal the neurological and psychological mechanisms behind this: depression weakens the social and empathic abilities that allow people to reach out for help, which means that the disorder driving the mental health crisis also actively destroys the fundamental emotions needed to seek recovery. Their finding that “social cognitive performance has been found to be inversely correlated with depression severity” (Kupferberg and Hasler 100631) means that the more a person needs community support, the less likely they are to seek and therefore receive it, creating a self-harming cycle that invites isolation and despair. The failure to reach out is often overlooked as social apathy, but in reality, it is a symptom of the disorder itself, not a character flaw. This supports Obama's call to action towards the responsibilities of the communities to bridge the gaps and include the isolated people to get them back involved, because most of them are simply falling victim to a neurological impairment. The absence of community structures that are willing to reach out is an active driver of the rising suicide rates and death tolls

Mahmood, Asos, et al. "Disparities in access to mental health services among children diagnosed with anxiety and depression in the United States." *Community mental health journal* 60.8 (2024): 1532-1546.

Asos Mahmood and colleagues examined barriers to mental health services and their access for children and adolescents with clinically diagnosed depression and anxiety, using data from the 2020-2021 National Survey of Children's Health. The sample they used includes 6,655 children, all between the ages of six and seventeen, and they found in their research that approximately 50.8 percent of caregivers reported obtaining and providing mental health services to their children was moderately difficult, very difficult, or impossible. The study displayed the alarming trends regarding the rising mental health crisis: rates of anxiety and depression among children have increased by 29 percent and 27 percent, respectively, between 2016 and 2020. Mahmood et al. identify the factors leading to the lack of access to youth mental healthcare: cost,

inadequate insurance coverage, and shortages in mental health providers. These have been labeled as the primary barriers to access. Mahmood et al. note that poverty is strongly associated with both higher rates of mental health conditions and reduced access to needed care, creating a double barrier that falls on the already most marginalized communities. The authors bring up the declaration of a state of emergency in children and adolescent mental health by national health organizations, as well as calling for systemic models as the most promising and efficient path towards creating equal access.

How does the lack of access to basic healthcare impact long-term depression rates?

Mahmood and colleagues' research is significant in understanding why depression and suicide rates continue to escalate in America, even as awareness continues to grow. The rising awareness alongside rising death rates is partially explained by this source; awareness of an issue without access to a solution produces a generation of young adults who know they have suffered and know that help exists, but are unable to reach it. Obama's eulogy speaks to the neglect of the marginalized communities where institutional support has been withdrawn or is very slim for individuals who are struggling to cope without tools or resources. Mahmood et al. document the consequences within the communities that experience this on the level of children's mental health. They found that half of families with a child diagnosed with anxiety or depression are unable to access care, displaying the urgency to fix the structural emergency. Sadly, childhood depression that goes untreated does not resolve on its own, but rather gets blocked out and multiplied along with adult despair, substance abuse, and suicidal ideation that have all become identifying features of the current national crisis. Mahmood et al. confirm that childhood rates of anxiety and depression "increased by 29% and 27% respectively between 2016 and 2020" (Mahmood et al. 1534), a trend that, if left without intervention, aligns directly with the worsening adult mental health and suicide statistics that led to the events that prompted Obama's eulogy. The source answers the small inquiry question with clear statements, evidence, and a state of urgency, and it supports the large inquiry question by demonstrating that the systemic failures of the American society – the unequal distribution of healthcare, the inadequacy of insurance coverage, and overall cost of mental health providers – are more than basic factors but the primary causes of the climbing depression and suicide rates Obama predicts.

Westberg, Katrin Häggström, et al. "Mental health problems among young people—a scoping review of help-seeking." *International journal of environmental research and public health* 19.3 (2022): 1430.

Katrin Häggström Westberg and colleagues conducted a review of literature published between the years 2010 and 2020 to organize young adults' experiences of seeking help for mental health problems, drawing from twelve studies from four major academic databases. The review found that young people feel unfamiliar and insecure about speaking about their mental health, frequently elongating their own suffering, minimizing their feelings as unimportant or not serious, as a way to avoid vulnerability with clinical strangers. Westberg et al. identify the terms stigma and shame as the most powerful and newly prominent themes in the selected literature, noting that young people follow the cultural assumptions of the modern world: asking for help makes you appear weak to your peers. Individuals within the studies reported intense fear of social consequences, ridicule, and loss of social image if their mental health struggles were to become public. The review documents that especially young men felt constrained to masculine

ideals of carrying yourself with strength and self-reliance, making displaying your emotional needs forbidden. The authors also find that young adults fear mistrust of mental health providers through the fear of breaching confidentiality, or an adult finding out, due to needing to commute to an institution. All these factors have created a layered obstacle course between a struggling young person and the help they need.

Does the fear of others' opinions affect the psychological safety of communities?

Westberg and colleagues' review built upon research from earlier theories, but this 2022 research has solidified a key idea: the fear of disrupting one's social image is a powerful enough force to override the survival instinct of young people who are suffering from depression and suicidal ideation, and Westberg et al. discovered the internal cognitive functions from the young person's perspective. When communities fail to signal that vulnerability is safe – when schools, families, neighborhoods, etc. lead with unspoken rules that associate individual emotional struggle with weakness or shame – young people read those signals and adjust accordingly, hiding their mental health problems and abandoning the pathways back to recovery. The review's finding that young people described “fear of social consequences, ridicule and a longing to fit in” as primary reasons for hiding their struggle (Westberg et al. 1430) aligns with the psychological safety deficits that Obama's eulogy touches on as a defining feature of American life. Psychological safety – the shared sense that it is acceptable to be honest with one another about their mental struggles – is not created by public awareness alone; it is created by the implementation of daily practices and attitudes within the communities, families, and institutions. When this sense of safety is unavailable, young people tend to suffer alone, and the silence they surround themselves with becomes one of the most common trends leading to depression, self-harm, and suicide, which Obama mourns as the new modern national crisis.